ETERNAL REPOS

TRIBUTE IN MOTION

Extending Care Beyond the Clinical

A hospital partnership proposal

DELHI NCR · PRIVATE & CONFIDENTIAL



Every farewell deserves the same care, dignity and thoughtfulness as every beginning.

This thought gave birth to Eternal Repos.

We prepare for life's beginnings with care, yet its final journey often leaves families overwhelmed with grief and responsibilities.

From the first call to the final rites, we stand beside families — managing arrangements, documentation, transportation and rituals with sensitivity and respect.

OUR PURPOSE

To honour the departed and bring care, clarity and peace of mind to the families they leave behind.



The hour your hospital never gets to design.

Clinical care concludes. The family still needs **guidance**, **coordination** and the **respectful** movement of their loved one.

*The **last interaction with a hospital** can remain with a family for years.*

Five handoffs. No one owns the sequence.

THE **CALL**	THE **HANDLING**	THE **WORDS**	THE **RECORD**	THE **WEEKS AFTER**
Unverified	**Unauditable**	**Untrained**	**Unrecorded**	**Unattended**
Whoever **answers**, or whoever is at the gate.	No written **hygiene** or handling protocol exists.	Tone is **temperament**, not **discipline**.	The family repeats everything to everyone.	**Service ends**. The paperwork does not.

*A system gap, not a hospital failing. These breaks sit between institutions — **which is exactly why none of them is anyone's job today.***

One accountable partner for the whole sequence.

ONE CONTINUOUS, ACCOUNTABLE SEQUENCE

1 Hospital

Clinical and **administrative** process concludes, unchanged.

2 Family

Informed an option exists. The choice stays theirs.

3 Final journey

Coordinated end to end, to the family's wishes.

WHO WE ARE

India’s First purpose-built Ceremonial Mobility Brand.

Not moving a person from one place to another - bringing **respect**, **coordination** and **consistency** to a transition that is usually left undesigned.

FOUNDED BY	**Dr. Shaily Sharma**
BUILT BY	**Azimuth Business on Wheels**
ENTITY	**Consurgo Ventures Pvt. Ltd.**
SERVING	**Delhi NCR**

RECOGNISED BY ET NOW, 2026

“Excellence in Creating Dignified Farewell Journeys”

ETNow.in Business Conclave
& Awards 2026, West Edition

SERVICE SCOPE

Five areas of care. A Hospital Partnership involves only the first.

Immediate **Relief** & **Transport**	**Serene** Gatherings	Natural **Beauty** & **Reverence**	Empathic **Care** & **Keepsakes**	Reassuring **Peace** of Mind

The rest remain available to the family, at their discretion.

From Transportation to Ceremonial Mobility.

CONVENTIONAL	ETERNAL REPOS
Utility first	**Dignity first**
Presentation varies	**Presentation is Standardised**
A vehicle that carries	**A space that holds**



The Sanctuary — design render

HOW IT WOULD WORK

Three points of hospital contact. That is all.

1

Your internal process, unchanged.

2

Family told support exists.

3

With consent, we are called.

4

We coordinate with the family.

5

Agreed transfer protocol.

6

Dignified handover.

7

Journey continues to their wishes.

Hospital involvement — **Everything else happens between us and the family.**

WHAT CHANGES

A better final interaction - for both.

FOR THE HOSPITAL

- A difficult transition gets structured support
- Staff stop brokering with outside providers
- One accountable partner, one protocol
- Your philosophy of care extends to the last hour

FOR THE FAMILY

- One dependable point of coordination
- Clarity about what happens next
- No time spent verifying strangers
- Room to be with family, not logistics

*Choice stays with the **family**. Presented as an option — **never a default, never a requirement.***

WAYS TO WORK TOGETHER

Three models. You choose the depth.

Empanelled partner

An approved **final-journey** option available to families.

DEEPEST

Family choice / referral

Information offered alongside other options. The **family engages** directly.

LIGHTEST

Integrated access

A dedicated **line**, **QR access** or an **assistance card** - designed with you.

TO BE SHAPED

The final structure follows your policy and applicable regulation. We are not asking you to choose today.

THE ASK

Six things. None require new budget.

1. **Recognition** as one professional option available to families
2. A named hospital coordinator for **operational alignment**
3. An agreed transfer protocol - **access**, **documents**, **handover**
4. Discreet family **information** where policy permits
5. A short **orientation** for the relevant teams
6. **Permission** to begin with a defined pilot

No referral fee. No commission. No exclusivity. Pricing published to the family in writing.

JUST AS IMPORTANT

Partnership without operational burden.

YOUR STAFF WILL NOT BE ASKED TO

- Sell or promote our **services**
- Persuade families towards any **choice**
- Handle **commercial** discussions
- Manage our **customer** service
- Carry responsibility for our **operations**

THE HOSPITAL'S ROLE, IN FULL

Making families aware that a professional option exists.

WHAT SETS US APART

Why a hospital should choose us.

1. **Complete Support**
   Every arrangement handled through a single point of contact.

2. **All Faiths Respected**
   Hindu, Muslim, Sikh, Jain, Buddhist & Christian rites conducted with care.

3. **Verified Network**
   Surveyed cremation grounds and vetted vendors across Delhi NCR.

4. **Dignified Handling**
   Trained staff follow strict body-handling and cremation protocols.

5. **Intercity & Repatriation**
   Transfer of the deceased to other cities and countries.

6. **Post-Cremation Care**
   Rituals, prayer meetings and memorial keepsakes.

7. **Compassionate Assistance**
   An empathetic team available whenever you need us.

8. **Transparent Pricing**
   Costs explained in advance, with no hidden charges.

9. **ISO-Certified Quality**
   ISO-certified vehicles & services; handling certification in progress.

One accountable partner for every faith, every family, every final journey.

WHAT WE ARE PROPOSING

A 60–90 day pilot, reviewed monthly.

1. Name a coordinator
2. Agree the SOP
3. Brief the teams
4. Open access
5. Review at 30 and 60

Reviewed on **enquiries**, **families assisted**, **coordination quality**, and **feedback** from both your staff and the families.

We are not proposing **targets** before we have evidence.

Because dignity should remain part of care until the very end.

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